National Cancer Control Programme (NCCP) 00551 · clinician draft

Nivolumab 3 mg/kg + ipilimumab 1 mg/kg — advanced melanoma

The treatment this guide covers NCCP identifies nivolumab 3 mg/kg plus ipilimumab 1 mg/kg for melanoma as an unlicensed dosing posology in Ireland, despite reimbursement. Your clinician should explain this, the available alternatives and uncertainties, document informed consent under hospital policy, and follow the hospital Drug and Therapeutics Committee or equivalent acknowledgement process. CheckMate 511 primarily compared safety; it did not prove that this schedule has equal survival benefit to nivolumab 1 mg/kg plus ipilimumab 3 mg/kg.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Melanoma · combination induction · up to four doses

Active medicines: Nivolumab + Ipilimumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once daily	Day 1, if prescribed under the low-risk plan Current NCIS V8 Table 1 p4. Review QT, electrolytes, constipation, hepatic dosing and interactions; this reference requires an individual prescription.
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days; duration to be prescribed Review contraindications, renal function, interactions and extrapyramidal adverse effects.

Additional phase-specific support

- The medicine list is a source-based low-risk reference for an individual consultant-approved/local prescription; it is separate from infusion-reaction premedication and immune-toxicity steroid treatment.
- National Cancer Control Programme (NCCP) classifies the combination phase as low emetogenic risk and nivolumab alone as minimal risk. Anti-sickness treatment is individual. Routine infusion premedication is not usually required; a previous reaction may alter that plan. Steroid or hormone treatment for immune effects must have a written follow-up plan.
- Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, with symptom review during combination and maintenance phases. Cortisol and other organ tests are added when indicated. Recheck route, dose, interval and the maintenance start date when phases change.
- Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.
- Use effective contraception during treatment and for at least five months after the final nivolumab dose. Discuss fertility and breastfeeding before starting and tell the team promptly if pregnancy is possible. Keep the last-dose date on the treatment record.
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Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00551 v7a · p4 · NCCP NCIS medical-oncology antiemetics V8, Table 1 p4, low risk

Sources checked 2026-11-06 · SACTCheck v0.84.42 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Melanoma · nivolumab maintenance

Active medicines: Nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Nivolumab maintenance has minimal emetogenic risk; no automatic low-risk induction antiemetic prescription carries over. Routine infusion premedication is not usually required; prior reactions and local policy govern individual support.
- Blood count, kidney/liver tests, glucose and thyroid tests before every cycle, with symptom review during combination and maintenance phases. Cortisol and other organ tests are added when indicated. Recheck route, dose, interval and the maintenance start date when phases change.

- Immunotherapy can inflame healthy organs during treatment or after it stops. Contact the oncology team promptly for new or worsening diarrhoea, blood or mucus in stools, abdominal pain, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell every treating clinician that you are receiving or have received immunotherapy.
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- If a dose is delayed or held, obtain a revised written plan. The first two-weekly maintenance dose starts three weeks after the last combination dose; the first four-weekly dose starts six weeks afterwards. Do not calculate a different start from the maintenance interval alone. Earlier immune toxicity and permanent-stop decisions still apply.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00551 v7a · NCCP 00551 v7a supportive-care section; current product information for medicine-specific precautions

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